

Coverage of obesity drugs in Medicaid: what it did, who it reaches, what it could cost

Insight deck · public aggregate data, 2018 to early 2026

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The answer: coverage was associated with a steady rise in prescriptions, and a program of 1 million enrollees would have a net cost of about \$161.3 million over five years

12.2

estimated additional prescriptions per 1,000 enrollees per quarter
95% CI 8.4 to 16.0

1.3 to 17.2

per 1,000 enrollees per quarter, first quarter to eighth (market growth included)

\$55.6 to 369.4 million

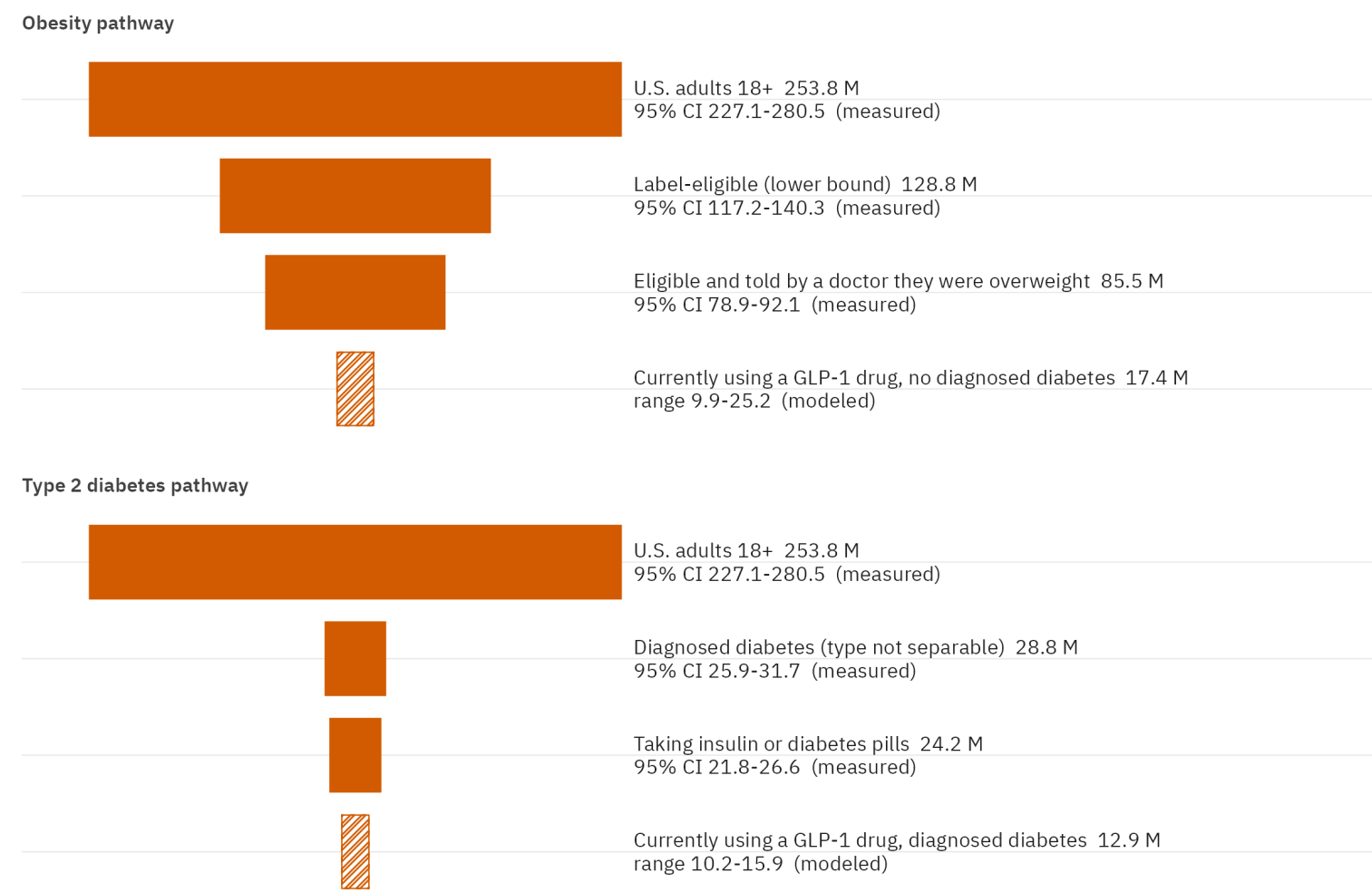
five-year net cost across scenarios, 1 million enrollees

\$2.69

net per enrollee per month, central case (rebate 51.2% is assumed)

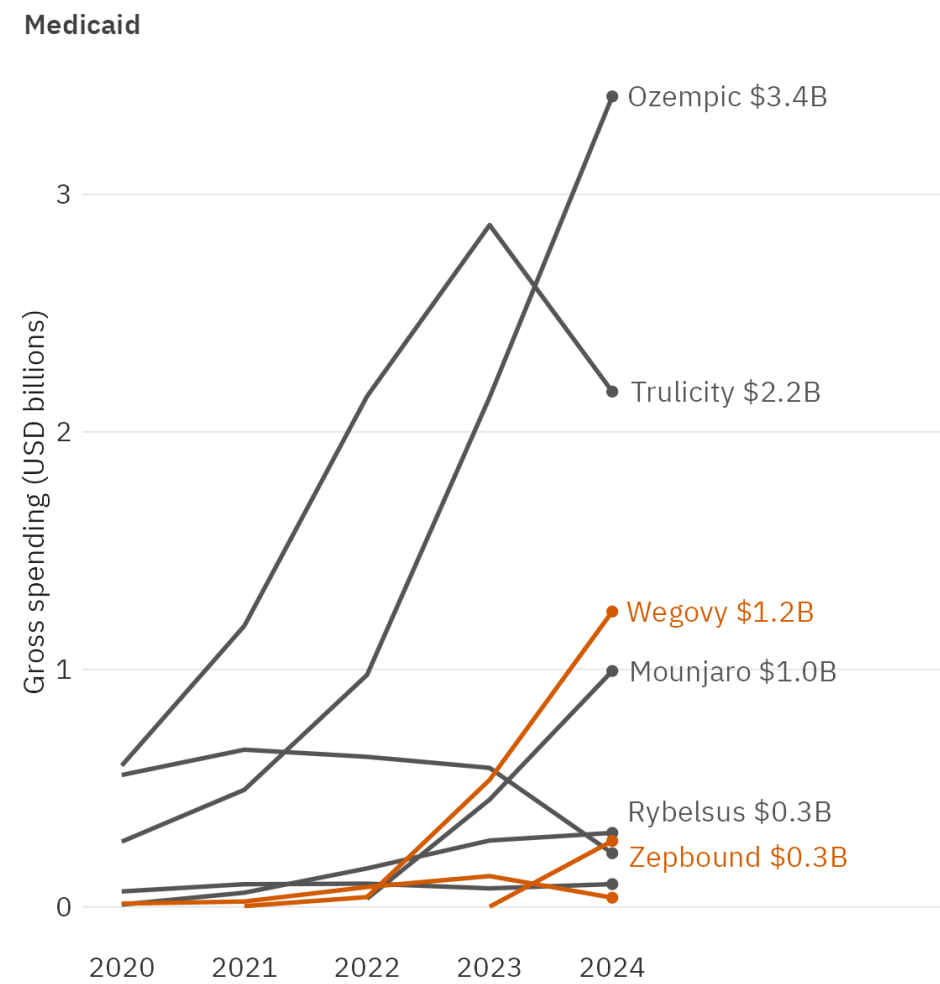
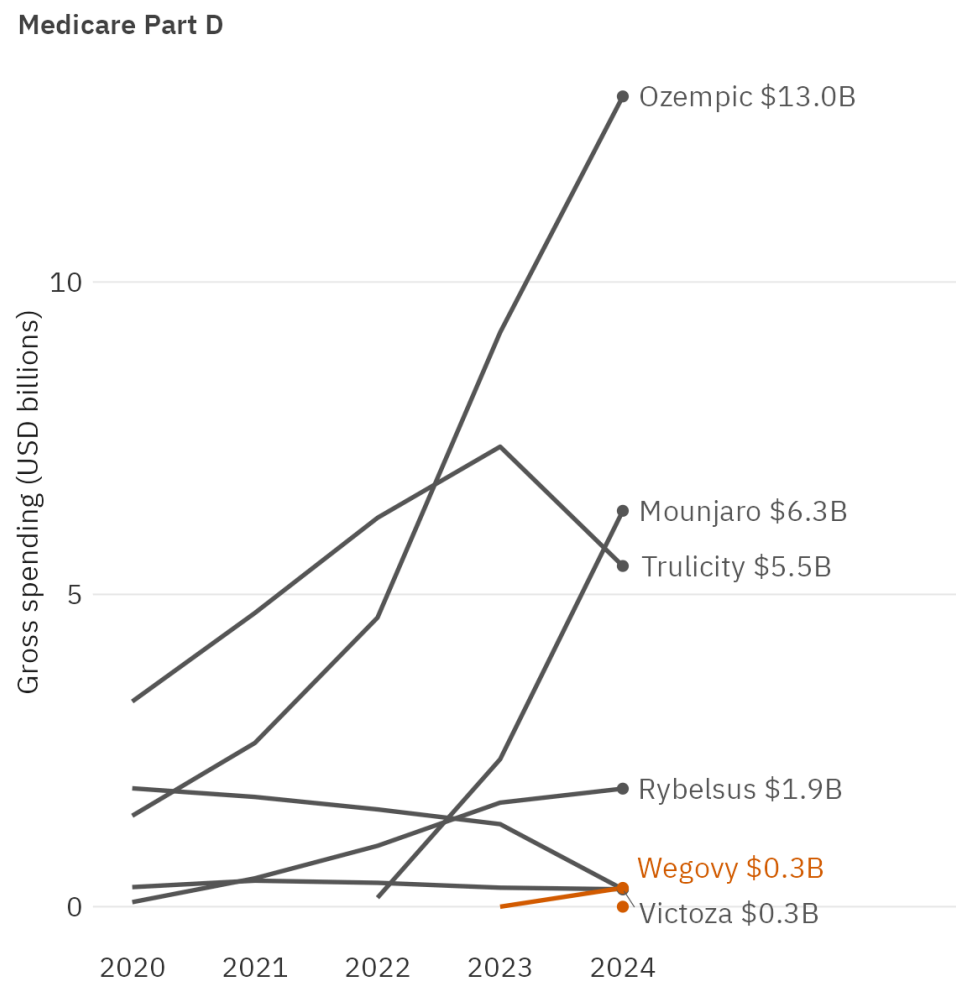
Sources: CMS State Drug Utilization Data and Medicaid enrollment (Module C); Module E budget model. Net of an assumed rebate; SDUD amounts are gross. Scenarios, not forecasts.

Market today: about half of U.S. adults meet the label criteria, and about 30.4 million already use a GLP-1 drug



Source: NHANES 2021-2023 (measured) and KFF poll, February 24 to March 2, 2026 (modeled). Eligible is a lower bound: 128.8 million adults (95% CI 117.2 to 140.3 million).

Market today: gross spending on GLP-1 and GIP products reached \$27.5 billion in Part D and \$8.8 billion in Medicaid in 2024, before rebates

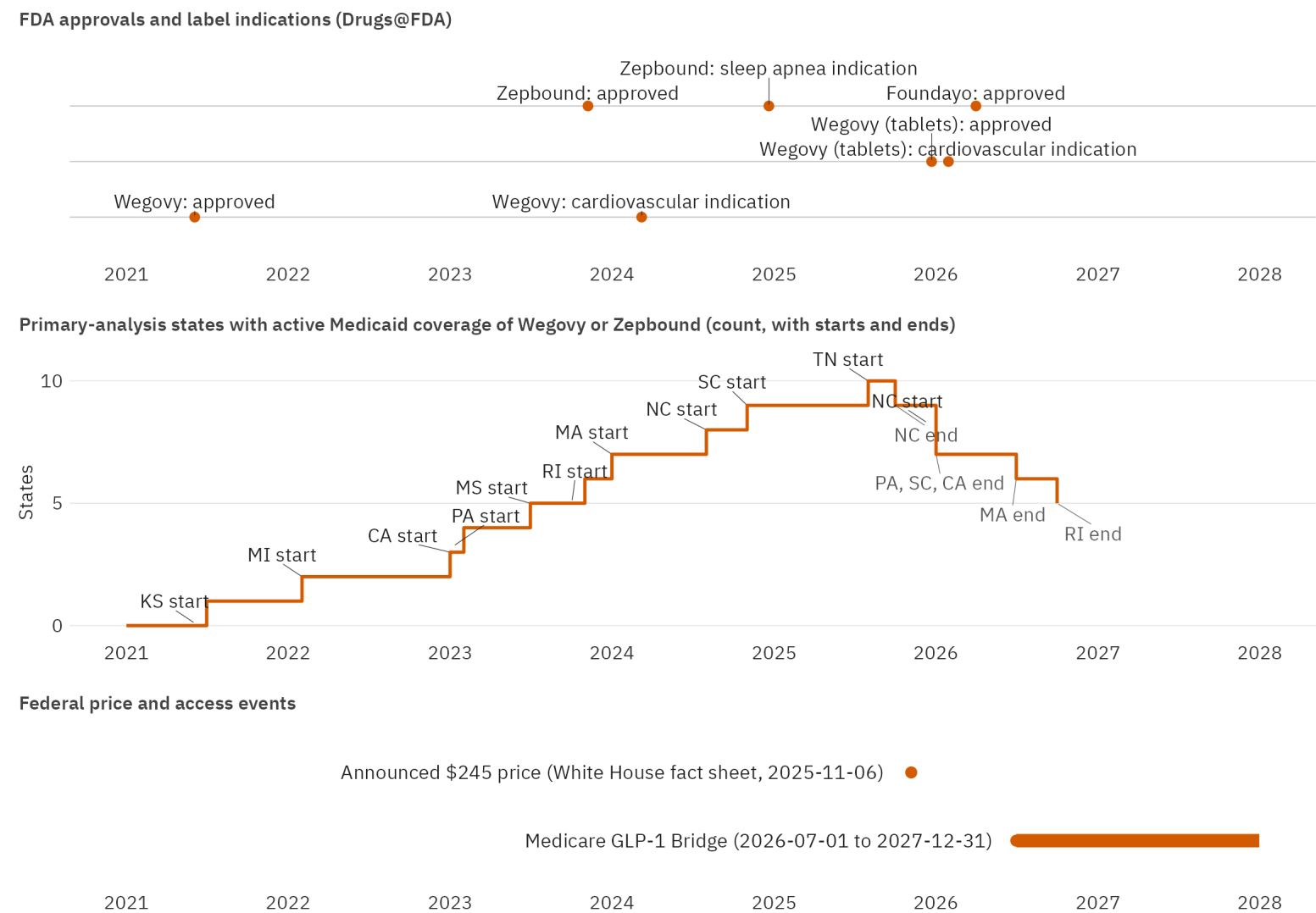


Landscape: injectables, new oral options, a Medicare Bridge and a \$245 price are arriving while some states end coverage

- **Products.** Wegovy (injection and tablets), Zepbound (KwikPen) and Foundayo (tablets) are covered under the Medicare GLP-1 Bridge (CMS prescriber page, June 2026).
- **Medicare GLP-1 Bridge.** A short-term demonstration, July 1, 2026 to December 31, 2027, with a \$50 copay (CMS).
- **Price.** The White House (November 2025) announced a Medicare price of \$245 a month for these drugs and said state Medicaid programs will have access at those prices.
- **State coverage is changing:** several states ended or paused coverage in late 2025 and early 2026 (timeline on the next slide).
- **Pipeline.** 634 registered studies of the in-scope ingredients list obesity; 59 are ongoing Phase 3, and oral orforglipron has 14 Phase 3 studies (ClinicalTrials.gov; other ingredients not counted).

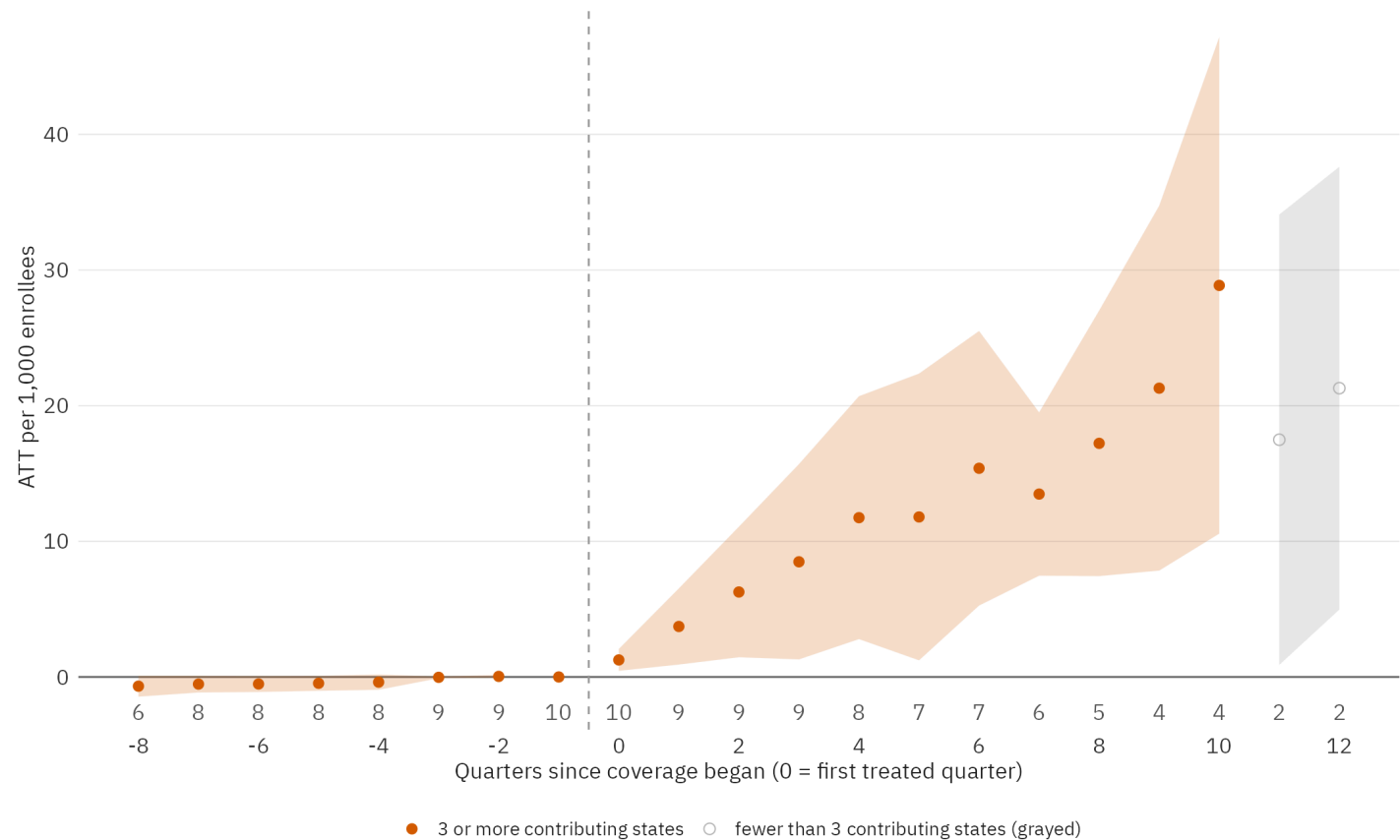
Sources: CMS Medicare GLP-1 Bridge pages (saved under docs/sources); White House fact sheet, November 2025; state Medicaid documents; ClinicalTrials.gov (registry last updated 2026-10-02).

Timeline: label expansions, state coverage starts and ends, the \$245 announcement and the Medicare Bridge cluster in 2024 to 2026



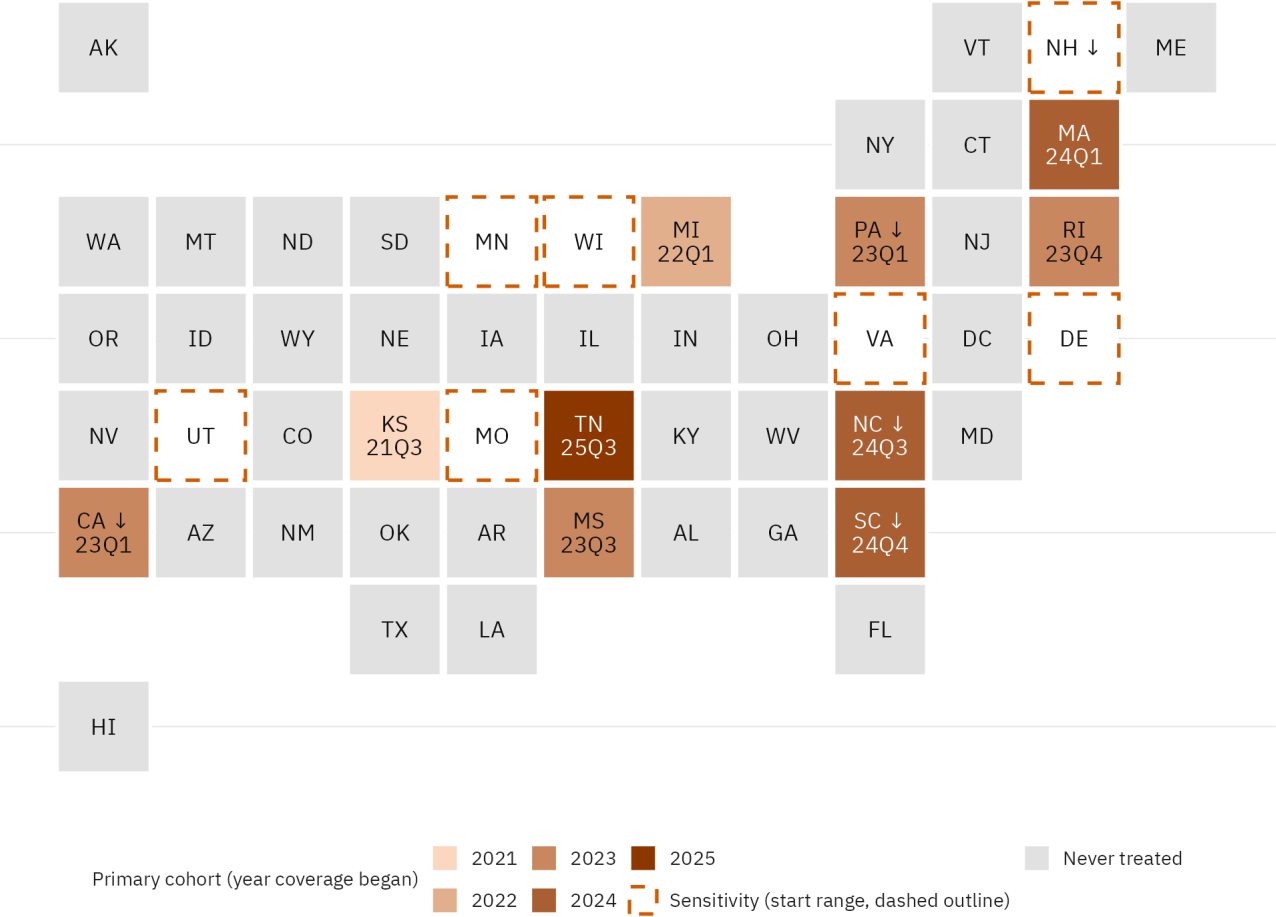
Sources: FDA Drugs@FDA labels; state Medicaid documents (primary-analysis states); White House fact sheet, November 2025; CMS Medicare GLP-1 Bridge pages.

Access: coverage was associated with an estimated 12.2 additional prescriptions per 1,000 enrollees per quarter, rising from 1.3 to 17.2 over eight quarters



Source: CMS SDUD and Medicaid enrollment; 10 covering and 34 never-covering jurisdictions. Placebo 0.4 (95% CI -0.1 to 0.8); other estimators and sensitivity analyses range 10.1 to 14.7. Part of the growth is national market growth.

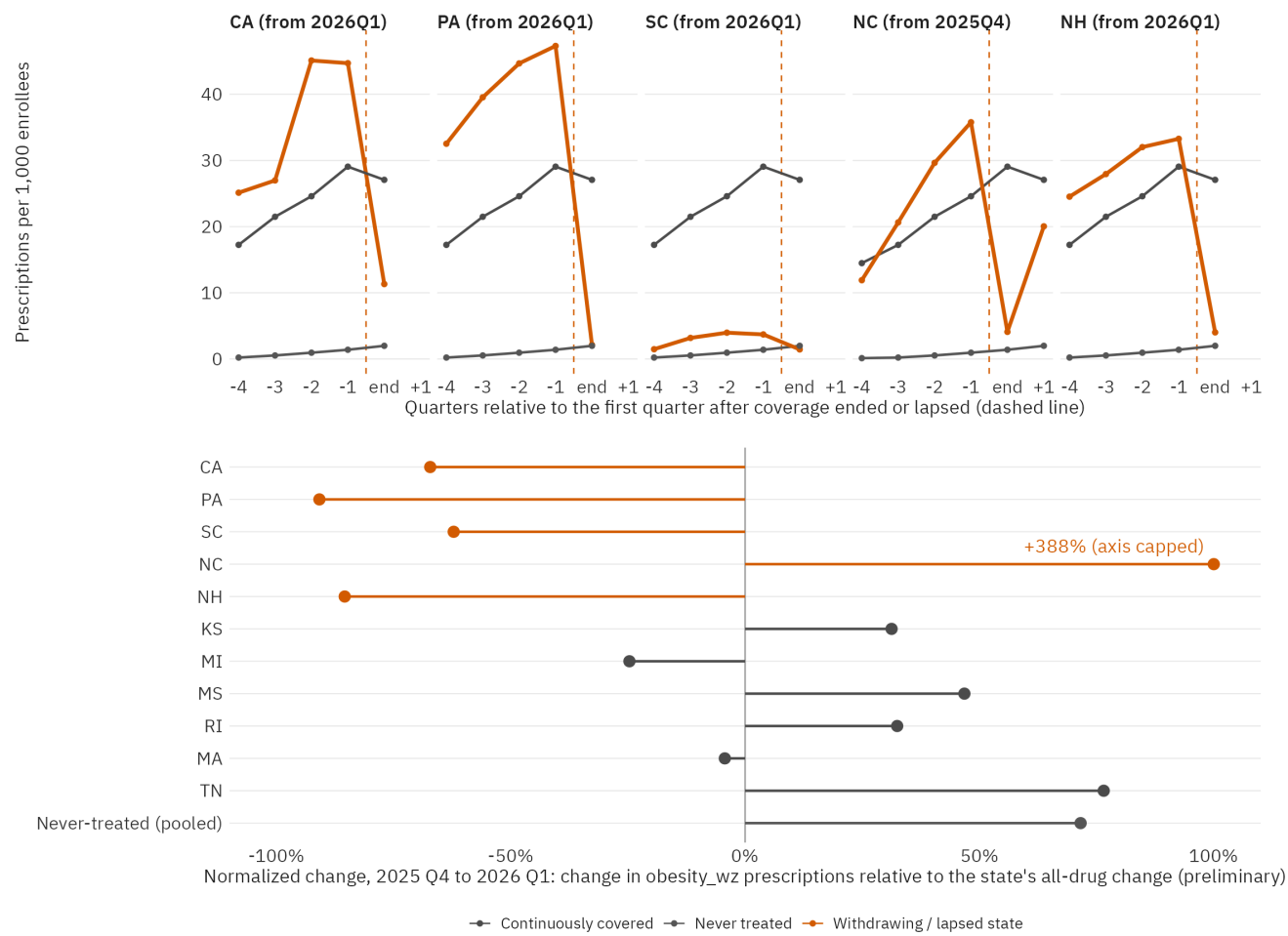
Access: 10 states form the primary analysis, and their prior authorization and BMI rules differ



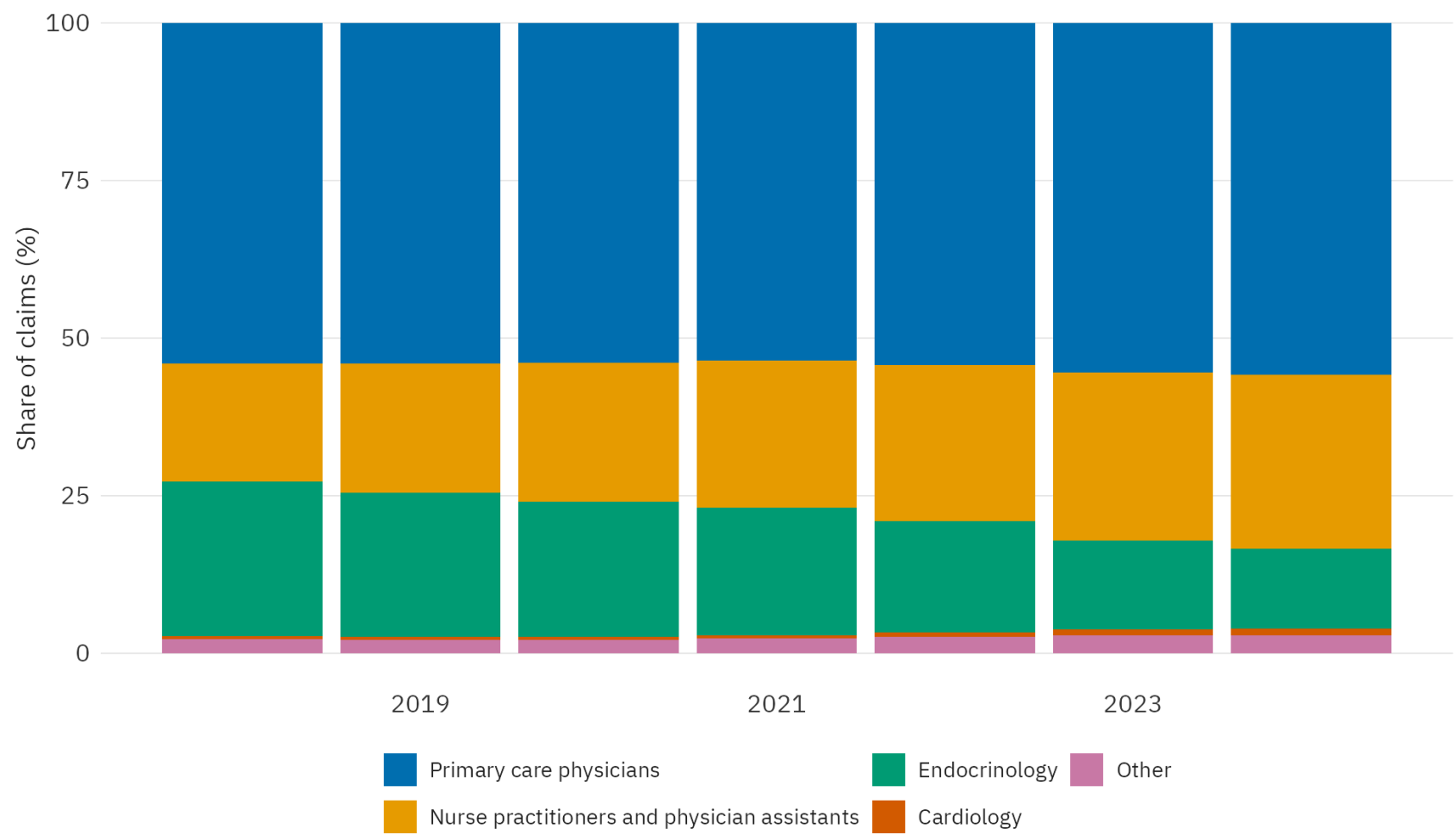
- **Prior authorization** is documented in 9 of 10 of the primary states; California's is not found in sourced documents.
- **BMI thresholds** are documented for 12 of the 17 covering states and step therapy for 13; the rules differ by state (criteria table in the report).
- **Timing.** 7 more states with uncertain start quarters appear only in a sensitivity analysis.

Source: state Medicaid documents and CMS approvals (docs/sources); criteria table: coverage_um_criteria.csv in the repository.

Access, detail: coverage operates through managed care (South Carolina, Rhode Island), and the first withdrawals show how fast it can reverse

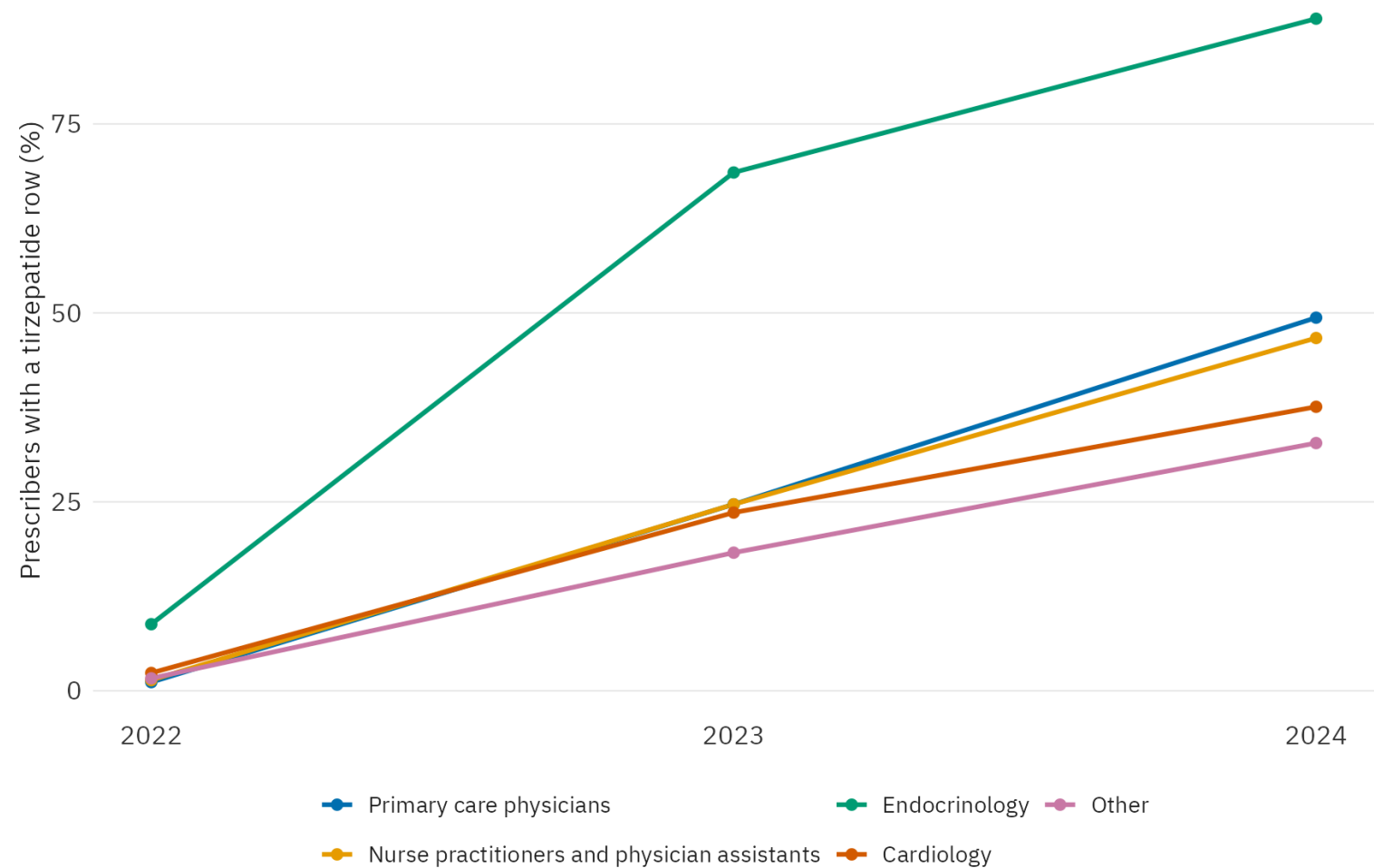


Prescribers: primary care and nurse practitioners and physician assistants wrote 55.8% and 27.7% of Part D incretin claims in 2024



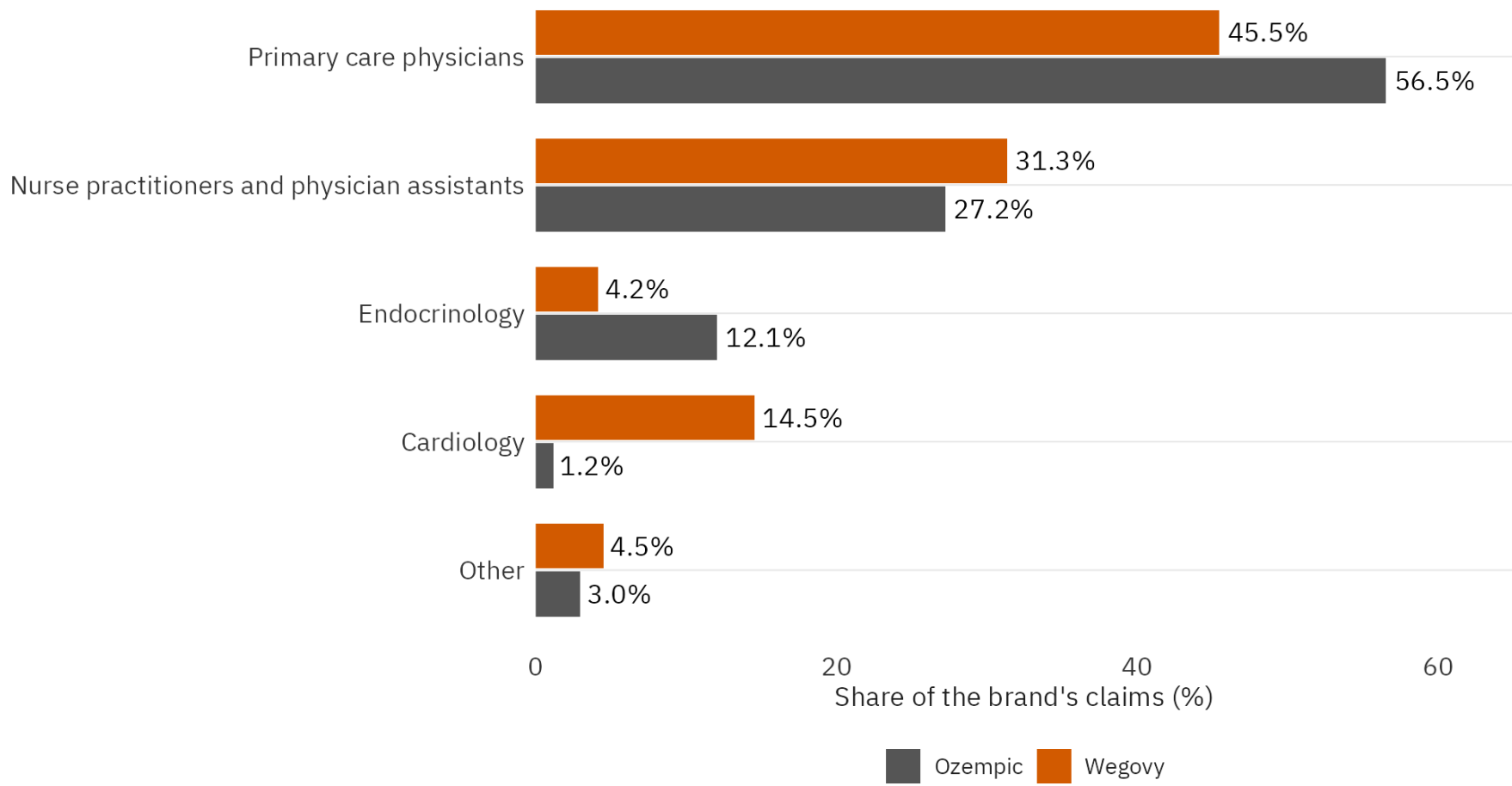
Source: CMS Medicare Part D Prescribers. Part D reflects diabetes and other covered uses, not obesity-brand adoption.

Prescribers: tirzepatide reached 48.6% of Part D incretin prescribers by 2024, from 1.6% in 2022



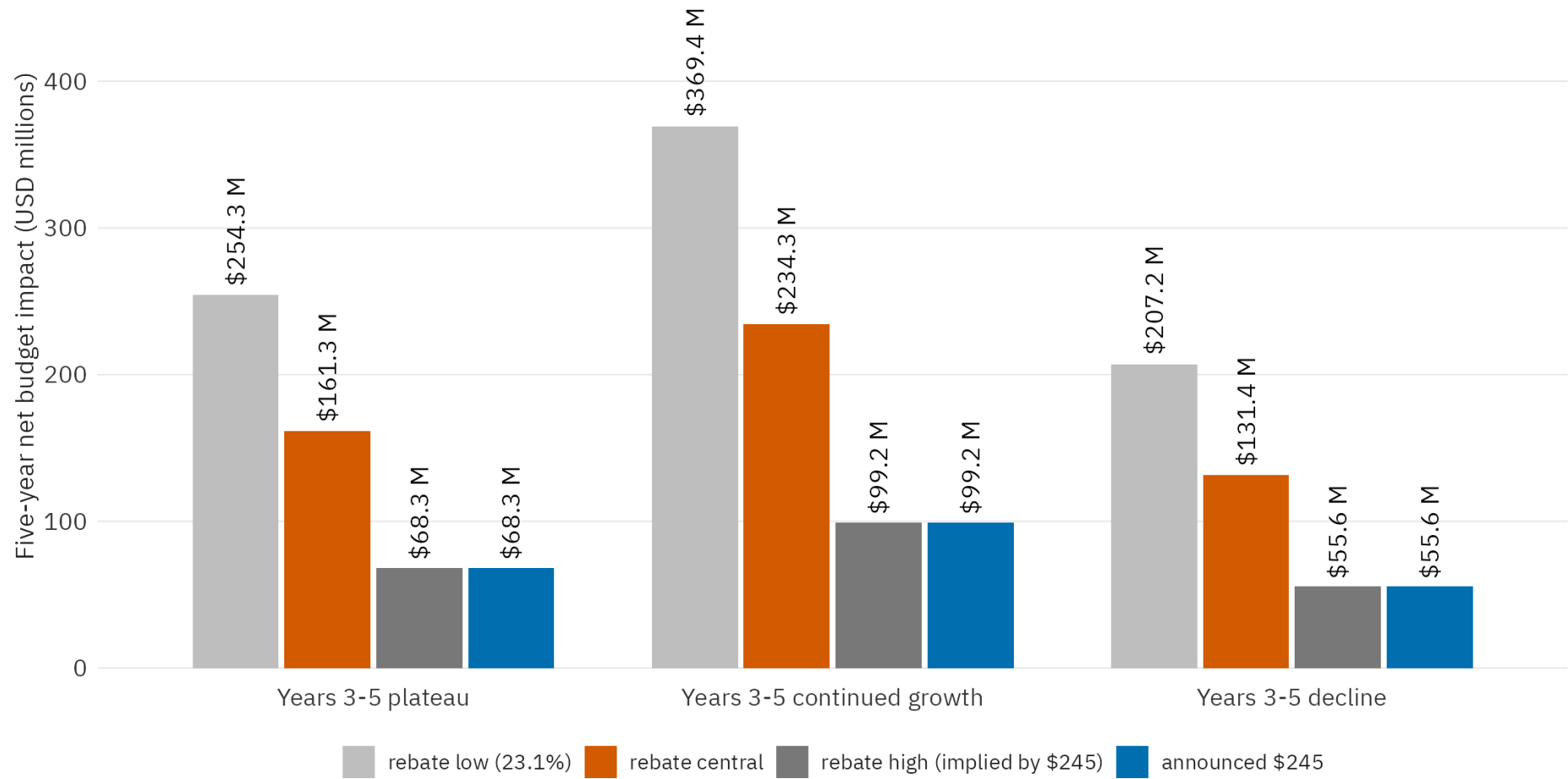
Source: CMS Medicare Part D Prescribers. Part D reflects diabetes and other covered uses, not obesity-brand adoption.

Prescribers: cardiology wrote 14.5% of Wegovy's Part D claims against 1.2% of Ozempic's, after the cardiovascular indication



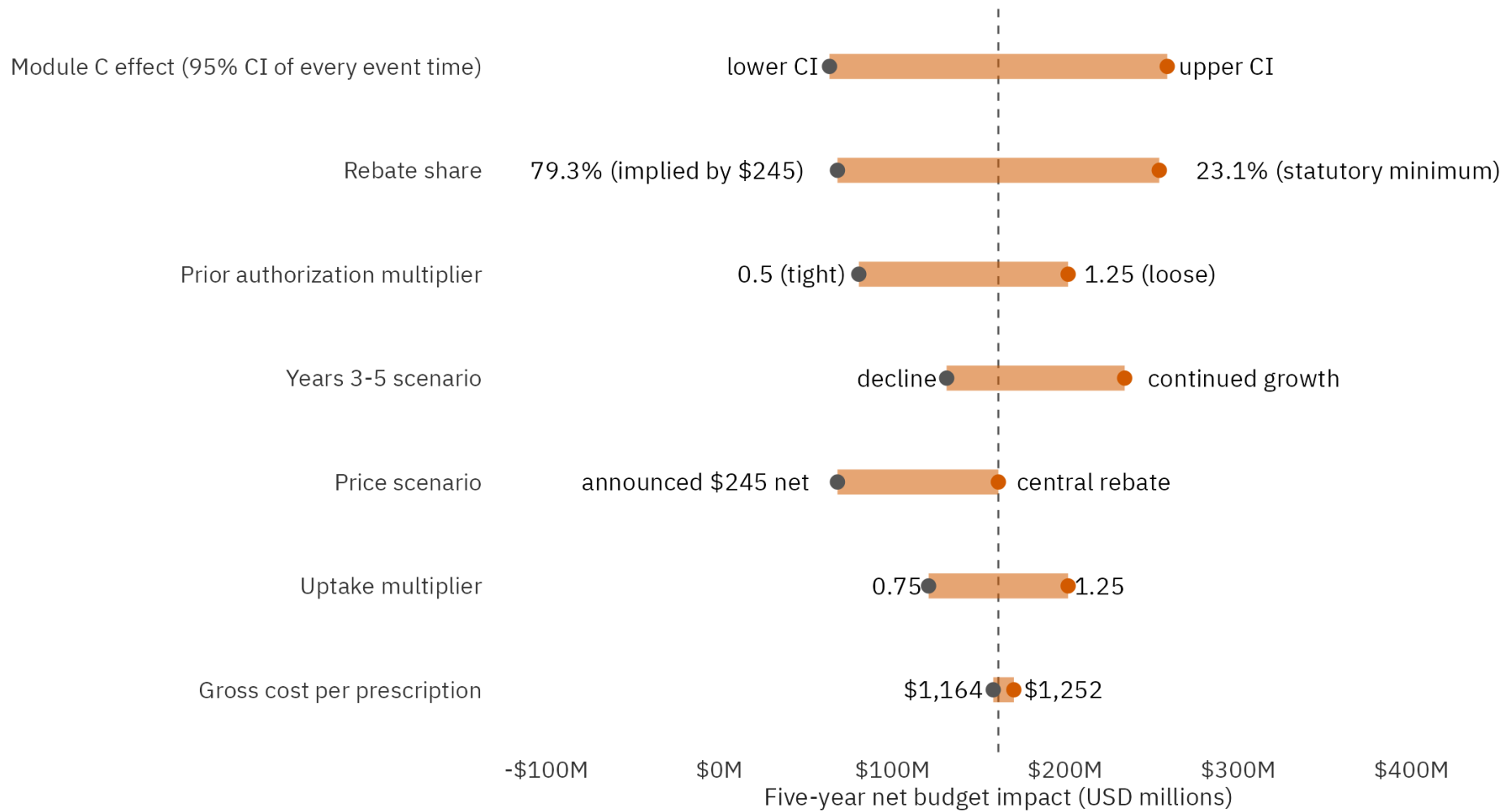
Source: CMS Medicare Part D Prescribers, 2024 (76,176 Wegovy claims from 3,956 prescribers). Part D reflects diabetes and other covered uses, not obesity-brand adoption.

Value: five-year net cost ranges from \$55.6 million to \$369.4 million for 1 million enrollees; the central case is \$161.3 million



Source: Module C effects, SDUD gross reimbursement, statutory rebate floor (42 U.S.C. 1396r-8), White House fact sheet. Rebates, prior authorization and years 3-5 are assumptions; no medical cost offsets.

Value: the rebate and the uncertainty in the coverage effect move the answer most



Source: Module E one-way sensitivity. Central rebate 51.2% is the midpoint of 23.1% (statutory minimum) and 79.3% (implied by the \$245 price); actual net prices are confidential.

Value: per enrollee per month, a gross \$5.51 becomes \$2.69 net at the central rebate and \$1.14 at the announced price



Source: Module E. The rebate step is an assumption; the announced price is a program price, not a confirmed Medicaid net price.

Value context: trial weight loss of -7.5% to -20.9% sits beside a Medicaid net cost of \$2,511 per user per year (context only)

Trial	Arm	Weeks	Weight change, % (95% CI)	Gross per Rx, USD
STEP 1	semaglutide	68	-14.9 (arm CI not in abstract)	1,303
SURMOUNT-1	tirzepatide 5 mg	72	-15.0 (-15.9 to -14.2)	1,048
SURMOUNT-1	tirzepatide 10 mg	72	-19.5 (-20.4 to -18.5)	1,048
SURMOUNT-1	tirzepatide 15 mg	72	-20.9 (-21.8 to -19.9)	1,048
SURMOUNT-5	tirzepatide	72	-20.2 (-21.4 to -19.1)	1,048
SURMOUNT-5	semaglutide	72	-13.7 (-14.9 to -12.6)	1,303
ATTAIN-1	orforglipron 6 mg	72	-7.5 (-8.2 to -6.8)	not a Medicaid-covered obesity product in the SDUD window
ATTAIN-1	orforglipron 12 mg	72	-8.4 (-9.1 to -7.7)	not a Medicaid-covered obesity product in the SDUD window
ATTAIN-1	orforglipron 36 mg	72	-11.2 (-12.0 to -10.4)	not a Medicaid-covered obesity product in the SDUD window

Trials: published abstracts via PubMed (STEP 1, SURMOUNT-1, SURMOUNT-5, ATTAIN-1). Net cost per user per year \$2,511 (4.3 fills, MEPS; midpoint rebate, blended Wegovy and Zepbound) and \$6,943 per member-year of continuous treatment (12 fills, assumption). Different populations, durations and fills: no cost-effectiveness claim.

Recommendations: considerations for a brand or access lead, from the evidence above

Finding	So what	What a brand or access lead might consider
Coverage was associated with an estimated 12.2 additional prescriptions per 1,000 enrollees per quarter, growing over eight quarters	Uptake builds over about two years rather than at launch	Planning access support over a multi-year ramp instead of a launch spike
Coverage runs through managed care in South Carolina and Rhode Island	Decisions sit with managed-care organizations as well as state programs	Evidence and criteria materials aimed at managed-care medical and pharmacy directors (to be tested in the research pack)
Coverage ended in California and Pennsylvania (-67.2% and -90.8%, preliminary)	Budget predictability decides whether coverage lasts	Showing a clear range of cost scenarios: \$2.69 per enrollee per month at the central rebate, \$1.14 at the announced price
Primary care and nurse practitioners and physician assistants wrote 55.8% and 27.7% of Part D incretin claims	The prescriber base is broad, not specialist	Weighing primary-care reach (Part D shows diabetes and other covered uses; confirm for obesity with other data)
The rebate is unknown and drives the range (\$55.6 to \$369.4 million)	Public data cannot settle net cost	Treating the budget range as a conversation starter and checking it against a payer's own net price

Appendix: data, methods and limits

- **Data.** CMS Medicaid State Drug Utilization Data and enrollment, Medicare Part D Prescribers, NADAC, NHANES, MEPS, KFF polls, FDA labels and state Medicaid documents; all public aggregate data. 54 dbt models, 126 tests.
- **Methods.** Callaway and Sant'Anna difference-in-differences (staggered adoption) with imputation of suppressed cells; survey-weighted NHANES estimates; ISPOR-style budget impact model with a probabilistic sensitivity analysis.
- **Limits.** Gross, not net, of rebates; ten covering states; eligibility is a lower bound; Part D is not obesity use; scenarios, not forecasts.
- **Links.** Interactive budget model (<https://01a108a8-ecde-397c-5353-39196812b10c.share.connect.posit.cloud/>), report ([report/report.html](#)), plans and deviations ([analysis/outputs/deviations.md](#)), key numbers ([analysis/outputs/key_numbers.csv](#)), repository github.com/erickyegon/incretin-access-value.
- No company affiliation or endorsement. I used AI tools to help write code and documentation. The study design, methods and conclusions are my own, and I verified all results.